▼ VIEWPOINT

THE REAL TOLL OF COVID IN INDIA

Fresh Covid cases draw attention, but it's the undercounted deaths from 2021 that demand urgent scrutiny and systemic reform



BY K. SRINATH REDDY

Covid-19 is back in the news because of a rise in the number of persons testing positive in different states of India as well as the release of government data on deaths that occurred in 2021, when the country was devastated by the Delta variant of the SARS-CoV-2 virus. The recent spike in cases due to the less virulent JN.1 and BA.2 sub-lineages of Omicron is not a cause for panic, as no deaths have been reported. However, there is concern about the underestimation of Covid-related deaths in earlier official reports. Questions have been raised about why previous estimates were low and the marked interstate variations in the gap between reported Covid deaths and Covid-attributable excess deaths in 2021.

Official estimates of Covid-related deaths in 2020 and 2021 differed from modelled estimates of the World Health Organization (480,000 versus 4.7 million). Data released by the Registrar General of India (RGI) now reveal an excess of 2.1 million deaths in 2021 compared to 2020—a 26 per cent rise. The usual causes of death would not have substantially changed over the year, and road accident-related deaths would have actually decreased due to mobility restrictions. So, it is likely that many of the unexplained excess deaths were Covid-related. That would suggest that earlier official estimates were low. We must bear in mind that those estimates were made when usual data-gathering systems were under siege by a pandemic, which demanded action on many fronts.

While the fact of death is relatively easy to document, the cause of death may be difficult to ascertain due to poor certification practices of doctors, especially for out-of-hospital deaths. When there are associated conditions like cardiovascular disease or diabetes, death is often attributed to them even if Covid had supervened to cause serious complications. Healthcare providers, in public and private sectors, need better training to improve certification practices. Where certification is dubious, 'verbal autopsy' studies can be conducted to identify the probable cause of death with standardised questions. These can be performed by trained frontline health workers and women's self-help groups with guidance from staff and students of nearby medical colleges. Central and state governments must mandate, promote and institutionalise such partnerships between the district health systems and co-located medical colleges.

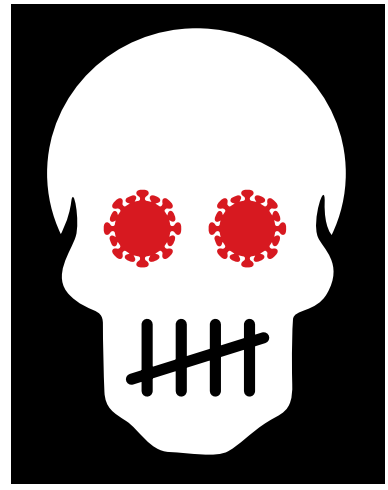


Illustration by TANMOY CHAKRABORTY

FROM THE OFFICIAL DATA SHOWING 2.1 MN EXCESS DEATHS IN 2021 OVER 2020, IT CAN BE DEDUCED THAT MANY WERE LIKELY DUE TO COVID

Getting the total death counts right is important too. Both the Civil Registration System (which gathers data on a continuous basis) and the Sample Registration System (which conducts periodic sample surveys) need to be strengthened through better training. Accuracy and timeliness can both be improved through the introduction of algorithmically guided digital registration methods. Mathematically estimating 'excess deaths' through year-to-year comparisons can help overcome ambiguities on the cause of death and fill data gaps. Even for measuring heat wave-related deaths, estimation of excess deaths is proving useful.

Judging by the recent RGI data, northern and western Indian states exhibited large data discrepancies, appearing to have underreported Covid deaths. Kerala was the best performer, with the least gap between reported Covid deaths and estimated excess deaths. The state would have drawn strength from a well-functioning primary care system, high population literacy, strong community engagement, active role of panchayats and NGOs during the pandemic, as well as gearing up of health system efficiency during the 2018 Nipah virus outbreak. Even as we debate the Covid numbers, we must strengthen community-connected health systems across India. ■

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